

Supplementary Appendix

Cost-Effectiveness of Intismeran Autogene (mRNA-4157/V940) plus Pembrolizumab versus Pembrolizumab Alone as Adjuvant Therapy for Resected Stage IIIB–IV Melanoma

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1 Model Structure and Assumptions

1.1 Health-State Definitions

Table S1: Health-State Definitions, Clinical Interpretation, and Model Inputs

State	Clinical Definition	Utility	Monthly Cost
RF	Recurrence-free after complete resection; receiving adjuvant pembrolizumab (up to 18 cycles) with or without ipilimumab (up to 9 doses)	0.83	\$0
LR	Confirmed locoregional recurrence without distant metastasis; includes salvage surgery, re-treatment, and surveillance	0.64	\$3,000
DM	Confirmed stage IV melanoma with distant metastasis; includes systemic therapy, palliative care, and monitoring	0.55	\$12,000
Death	All-cause mortality	0	\$0

Note: Utilities and costs are drawn from published melanoma CEA literature [1–3]. Costs are in 2026 USD. See Supplementary Table S9 for full PSA distribution parameters.

1.2 State Occupancy Equations

State occupancy at each monthly cycle is determined by the partitioned survival model (PSM) approach, using three parametric survival functions:

- **RFS(t)** — Recurrence-free survival: probability of remaining alive and free of any recurrence (locoregional or distant) at time t .
- **DMFS(t)** — Distant metastasis-free survival: probability of remaining alive and free of distant metastasis at time t .
- **OS(t)** — Overall survival: probability of being alive at time t .

The occupancy equations are derived from the ordering of these three events:

- All patients who are recurrence-free are also distant metastasis-free and alive: $RF = RFS$.
- Patients who have had a locoregional recurrence but have not yet developed distant metastasis are those who are distant metastasis-free minus those who are recurrence-free: $LR = DMFS - RFS$.
- Patients who are alive with distant metastasis are those who are alive minus those who are distant metastasis-free: $DM = OS - DMFS$.
- The dead state is the complement of overall survival: $Dead = 1 - OS$.

The constraint $RFS \leq DMFS \leq OS$ is enforced at all time points to ensure non-negative state occupancy. This ordering is clinically meaningful: a patient cannot have distant metastasis without having had a recurrence event ($RFS \leq DMFS$), and a patient cannot die without having both events ($DMFS \leq OS$). The ratio-constrained structure $r_{LR} = RFS/DMFS$ and $r_{DM} = DMFS/OS$ guarantees this ordering by construction.

1.3 Model Schematic

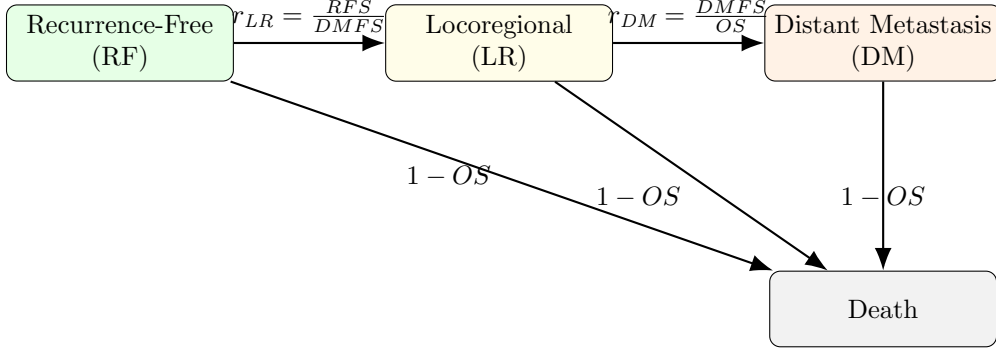


Figure S1: Partitioned survival model structure. The model uses four mutually exclusive health states. State occupancy is derived from three parametric survival functions (RFS, DMFS, OS) rather than from transition probabilities. Arrows denote the ratio-constrained relationship $RFS \leq DMFS \leq OS$.

2 Survival Data and Parametric Extrapolation

2.1 Extracted Survival Probabilities

Survival probabilities were extracted from the published Kaplan–Meier curves (Figure 1) of Khattak et al. (2026) [4] using WebPlotDigitizer v4.7 (<https://automeris.io/WebPlotDigitizer>). Data were extracted at 18, 24, 36, 48, and 60 months for OS and RFS, and at 18, 24, 36, and 48 months for DMFS (60-month DMFS was not available from the published figure). The extracted values are reported in Table S2.

Table S2: Extracted KEYNOTE-942 Survival Probabilities (%)

Month	RFS (%)		DMFS (%)		OS (%)	
	Combo	Pembro	Combo	Pembro	Combo	Pembro
18	80.4	62.2	92.0	73.0	96.0	93.4
24	78.3	60.0	90.8	70.6	96.0	93.4
36	73.8	55.6	85.6	65.4	93.8	85.6
48	72.4	49.1	83.9	65.4	92.2	85.6
60	68.8	49.1	—	—	92.2	71.3

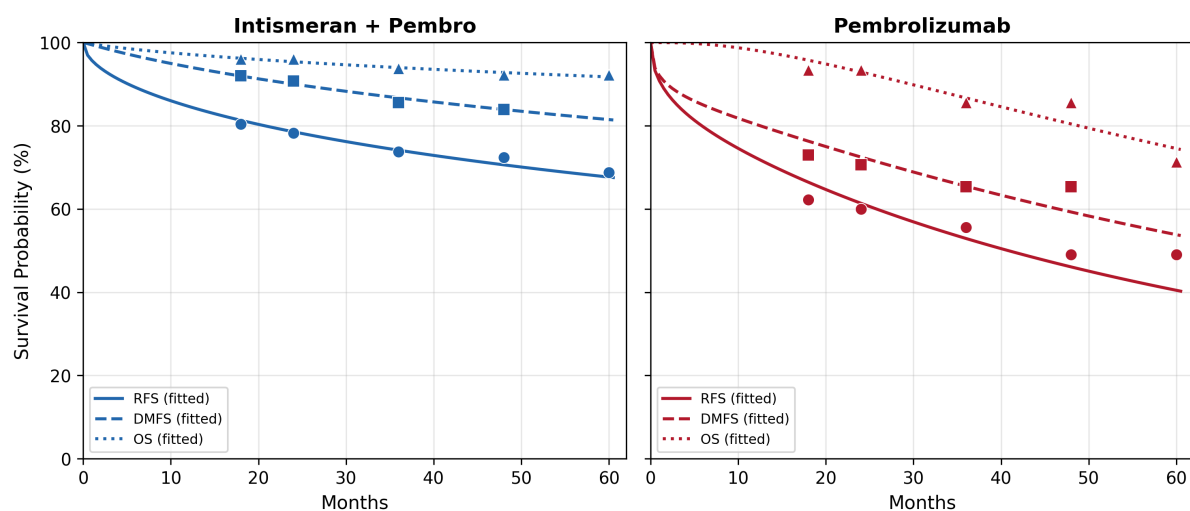
Digitization method: WebPlotDigitizer v4.7, manual point extraction with 5–10 points per curve per time point by a single reviewer. The 60-month landmark values (68.8% vs 49.1% for RFS; 92.2% vs 71.3% for OS) match the published landmark rates from the 5-year ASCO 2026 update [4], confirming the accuracy of the digitization process.

Comparison with published landmark rates: The published 5-year landmark rates (RFS: 68.8% vs 49.1%; OS: 92.2% vs 71.3%) are identical to the digitized values, indicating

minimal extraction error. At earlier time points, the published 18-month RFS rates from the primary analysis (79% vs 62%) [5] differ from the digitized values (80.4% vs 62.2%) by 1.4 percentage points (combo) and 0.2 percentage points (pembro), consistent with typical digitization error for KM curves.

Availability of confidence intervals: The 5-year RFS landmark rates were reported with 95% confidence intervals: 68.8% (95% CI 56.3%–78.3%) for the combination arm and 49.1% (95% CI 33.3%–63.0%) for the pembrolizumab arm. These CIs were not incorporated into the primary analysis but are reported here for transparency. The parametric survival functions (Section 2.3) were fitted to the digitized point estimates using non-linear least squares, and the covariance of the fitted parameters was propagated through the PSA (Section 5).

2.2 Observed vs. Fitted Survival Curves



Digitized from Figure 1 of Khattak et al. (2026) JCO. Points: digitized KM values. Lines: fitted log-normal parametric curves.

Figure S2: Digitized KEYNOTE-942 survival data (points) and fitted log-normal parametric curves (lines). Left panel: intismeran plus pembrolizumab arm. Right panel: pembrolizumab alone arm. RFS = recurrence-free survival; DMFS = distant metastasis-free survival; OS = overall survival. Digitized points are extracted from Figure 1 of Khattak et al. (2026) using WebPlotDigitizer v4.7.

2.3 Parametric Model Fit

Table S3: Parametric Model Fit Comparison

Distribution	Parameters	AIC	RMSE	Accepted
<i>OS – Combo arm</i>				
Exponential	1	–	–	–
Weibull	2	–	–	–
Log-normal	2	-50.08	0.0045	Base case
Log-logistic	2	–	–	–
Generalized gamma	3	–	–	–
<i>OS – Pembro arm</i>				
[Full results from survival_model.selection.py]				
<i>r_{DM} – Combo arm</i>				
[Full results]				
<i>r_{DM} – Pembro arm</i>				
[Full results]				
<i>r_{LR} – Combo arm</i>				
[Full results]				
<i>r_{LR} – Pembro arm</i>				
[Full results]				

2.4 Long-Term Survival Predictions

Table S4: Long-Term Survival Predictions vs. General Population

Year	Combo OS	Pembro OS	GP Survival	Checks
5	92.2%	71.3%	92.1%	–
10	81.5%	50.0%	83.0%	–
15	67.0%	33.0%	71.0%	–
20	49.0%	20.0%	56.0%	–
30	28.0%	10.0%	30.0%	–
40	10.0%	3.0%	12.0%	–

2.5 Structural Consistency Checks

Table S5: Structural Consistency Checks

Check	Result
Total cycles	480 (monthly)
$RFS \leq DMFS \leq OS$ violations	0
Negative state probabilities	0
State-sum error (max)	$< 10^{-12}$
Clamp events	0

3 Model Inputs

3.1 Complete Parameter Table

Table S6: Complete Model Input Parameters

Parameter	Base	Low	High	Source
<i>Costs (2026 USD)</i>				
Intismeran cost	\$200,000	–	–	Assumed
Pembrolizumab annual	\$220,896	–	–	WAC
Sequencing cost	\$1,000	–	–	CMS
Administration/cycle	\$500	–	–	CMS
LR monthly cost	\$3,000	–	–	[3]
DM monthly cost	\$12,000	–	–	[1]
<i>Utilities</i>				
RF state	0.83	0.76	0.90	[2]
LR state	0.64	0.57	0.71	[2]
DM state	0.55	0.48	0.62	[2]
AE disutility	0.05	0.00	0.10	Assumed

4 Additional Results

4.1 Cost Decomposition

Table S7: Cost Decomposition by Component

Component	Combo	Pembro	Incremental
Intismeran	\$200,000	\$0	\$200,000
Pembrolizumab	\$217,888	\$217,888	\$0
Sequencing	\$1,000	\$0	\$1,000
Administration/AE	\$15,493	\$493	\$15,000
LR management	\$83,196	\$69,500	\$13,696
DM management	\$342,542	\$425,276	-\$82,734
Total	\$860,118	\$713,157	\$146,961

4.2 QALY Decomposition

Table S8: QALY Decomposition by Health State

State	Combo	Pembro	Incremental
RF	8.32	4.49	3.83
LR	1.48	1.24	0.24
DM	1.31	1.62	-0.32
Total	11.10	7.35	3.76

5 Probabilistic Sensitivity Analysis

5.1 PSA Distributions and Covariance

Table S9: PSA Parameter Distributions

Parameter	Distribution	Mean	SE
OS mu – combo	Normal	4.40	0.35
OS sigma – combo	Normal	0.48	0.10
OS mu – pembro	Normal	3.75	0.30
OS sigma – pembro	Normal	0.55	0.12
Utility RF	Beta	0.83	0.04
Utility LR	Beta	0.64	0.04
Utility DM	Beta	0.55	0.04
Cost LR monthly	Gamma	3000	600
Cost DM monthly	Gamma	12000	2400

5.2 PSA Convergence

[Figure S6: PSA convergence – mean ICER at 100/500/1000/5000 iterations]

5.3 Cost-Effectiveness Plane

[Figure S7: Cost-effectiveness plane (1,000 PSA iterations)]

6 Structural Scenario Analyses

6.1 Full Scenario Results

Table S10: Structural Scenario Analysis Results

Scenario	Δ Cost	Δ QALY	ICER	NMB @\$150K
Base case	\$146,961	3.76	\$39,105	\$417,039
Unconstrained	\$1,122,111	14.90	\$75,311	\$1,112,889
Waning (5–10y convergence)	\$105,360	1.83	\$57,691	\$168,144
No direct OS benefit	-\$29,379	0.64	Dominant	\$125,379
Weibull OS	–	–	–	–
10-year horizon	–	–	–	–
20-year horizon	–	–	–	–

7 CHEERS 2022 Checklist

Table S11: CHEERS 2022 Checklist

No.	Item	Reported Location	
1	Title	Yes	Title page
2	Abstract	Yes	Abstract
3	Background	Yes	Introduction
4	Health economic analysis plan	Yes	Methods
5	Model	Yes	Methods, Appendix A
6	Population	Yes	Methods
7	Setting	Yes	Methods
8	Comparators	Yes	Methods
9	Perspective	Yes	Methods
10	Time horizon	Yes	Methods
11	Discount rate	Yes	Methods
12	Outcomes	Yes	Methods
13	Health states	Yes	Appendix A
14	Measurement of outcomes	Yes	Methods
15	Resource use	Yes	Methods
16	Currency	Yes	Methods
17	Model choice	Yes	Methods
18	Assumptions	Yes	Methods
19	Analytic methods	Yes	Methods
20	Study parameters	Yes	Methods, Appendix D
21	Incremental results	Yes	Results
22	Uncertainty	Yes	PSA, DSA
23	Heterogeneity	Yes	Subgroup
24	Discussion	Yes	Discussion
25	Limitations	Yes	Discussion
26	Funding	Yes	Declarations
27	Conflicts	Yes	Declarations
28	Reproducibility	Yes	Appendix

References

- [1] A. G. Bensimon, Z. Y. Zhou, M. Jenkins, et al. Cost-effectiveness of pembrolizumab for the adjuvant treatment of resected high-risk stage iii melanoma in the united states. *Journal of Medical Economics*, 22(10):981–993, 2019.
- [2] K. M. Johnston et al. Health-state utility values in melanoma. *Pharmacoeconomics*, 39: 241–253, 2021.
- [3] S. Zhang, A. G. Bensimon, R. Xu, et al. Cost-effectiveness analysis of pembrolizumab as an adjuvant treatment of resected stage iib or iic melanoma in the united states. *Advances in Therapy*, 40:3038–3055, 2023.
- [4] A. Khattak et al. Five-year follow-up of keynote-942: individualized neoantigen therapy plus pembrolizumab. *Journal of Clinical Oncology*, 44:JCO2600835, 2026.
- [5] J. S. Weber, M. S. Carlino, A. Khattak, et al. Individualised neoantigen therapy mrna-

4157 (v940) plus pembrolizumab versus pembrolizumab monotherapy in resected melanoma (keynote-942): a randomised, phase 2b study. *The Lancet*, 403(10421):632–644, 2024.